

		The court finds the delegation clause is not substantively unconscionable. (See <i>Armendariz v. Foundation Health Psychcare Services, Inc.</i> (2000) 24 Cal.4th 83, 114 [“both procedural and substantive unconscionability must both be present in order for a court to exercise its discretion to refuse to enforce a contract or clause under the doctrine of unconscionability.” (emphasis in original)].) Since both prerequisites have been satisfied, the delegation clause is enforceable and the arbitrator, not the court, must determine whether the Arbitration Agreement is unconscionable. Tentative Ruling: The motion is GRANTED. This action is stayed pending the outcome of arbitration. Moving party to give notice.
7	Quijano vs. Mission Hospital Regional Medical Center 2025-01517956	1. Demurrer to Amended Complaint 2. Case Management Conference The Defendant Mission Hospital Regional Medical Center dba Providence Mission Hospital Mission Viejo demurs to First Amended Complaint (“FAC”). <u>Legal Standard:</u> At the pleading stage, the Court must liberally construe the complaint, drawing all reasonable inferences in favor of Plaintiffs’ asserted claims. (<i>Liapes v. Facebook, Inc.</i> (2023) 95 Cal.App.5th 910, 919.) “A demurrer must dispose of an entire cause of action to be sustained. Thus, a court must overrule a demurrer to a cause of action if it is based on at least one viable theory of liability.” (<i>Thompson v. Spitzer</i> (2023) 90 Cal.App.5th 436, 451–452 [cleaned up].)

		“We not only treat the demurrer as admitting all material facts properly pleaded, but also give the complaint a reasonable interpretation, reading it as a whole and its parts in their context. If the complaint states a cause of action under any theory, regardless of the title under which the factual basis for relief is stated, that aspect of the complaint is good against a demurrer. [¶] The courts of this state have long since departed from holding a plaintiff strictly to the form of action that has been pleaded and instead have adopted the more flexible approach of examining the facts alleged to determine if a demurrer should be sustained. Where, as here, the demurrer is based on a claim that the pleading does not state facts sufficient to constitute a cause of action, if it appears that the plaintiff is entitled to any relief at the hands of the court against the defendants, the complaint will be held good, although the facts may not be clearly stated, or may be intermingled with a statement of other facts irrelevant to the cause of action shown, or although the plaintiff may demand relief to which he is not entitled under the facts alleged. [¶] In reviewing the legal sufficiency of a demurrer, we are not concerned with plaintiff’s ability to prove the allegations of the complaint, or the possible difficulties in making such proof.” (<i>Berry v. Frazier</i> (2023) 90 Cal.App.5th 1258, 1268 [cleaned up].) <u><i>Summary of Elements of Elder Abuse Claim:</i></u> Mission Hospital demurs to Plaintiffs’ second cause of action for Violations of Elder Abuse and Dependent Adult Civil Protection Act on the grounds that this claim is duplicative, uncertain, and fails to state sufficient facts. (Motion, p. 3.) “The Elder Abuse Act’s heightened remedies are available only in limited circumstances. A plaintiff must prove, by clear and convincing evidence, that a defendant is liable for either physical abuse under section 15610.63 or neglect under section 15610.57, and that the defendant committed the abuse with ‘recklessness,
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	oppression, fraud, or malice.’ (§ 15657.) Section 15610.57, in turn, provides two definitions of neglect. First, ‘[t]he negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise.’ (§ 15610.57, subd. (a)(1).) Second, ‘[t]he negligent failure of an elder or dependent adult to exercise that degree of self care that a reasonable person in a like position would exercise.’ (Id., subd. (a)(2).)” (<i>Winn v. Pioneer Medical Group, Inc.</i> (2016) 63 Cal.4th 148, 156.) “In order to obtain the remedies available in section 15657, a plaintiff must demonstrate by clear and convincing evidence that defendant is guilty of something more than negligence; he or she must show reckless, oppressive, fraudulent, or malicious conduct. The latter three categories involve ‘intentional,’ ‘willful,’ or ‘conscious’ wrongdoing of a ‘despicable’ or ‘injurious’ nature. (Civ.Code, § 3294, subd. (c); see also <i>College Hospital, Inc. v. Superior Court</i> (1994) 8 Cal.4th 704, 721, 34 Cal.Rptr.2d 898, 882 P.2d 894.) ‘Recklessness’ refers to a subjective state of culpability greater than simple negligence, which has been described as a ‘deliberate disregard’ of the ‘high degree of probability’ that an injury will occur (BAJI No. 12.77 [defining the ‘recklessness’ in the context of intentional infliction of emotional distress action]); see also Rest.2d Torts, § 500.) Recklessness, unlike negligence, involves more than ‘inadvertence, incompetence, unskillfulness, or a failure to take precautions’ but rather rises to the level of a ‘conscious choice of a course of action ... with knowledge of the serious danger to others involved in it.’” (<i>Delaney v. Baker</i> (1999) 20 Cal.4th 23, 31-32.) “Section 15657.2 can therefore be read as making clear that the acts proscribed by section 15657 do not include acts of simple professional negligence, but refer to forms of abuse or neglect performed with some state of culpability greater than mere negligence.” (<i>Id.</i> at 32.) <u>Application:</u>
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	In this cause of action, Plaintiffs allege decedent Jessica Quijano was a dependent adult admitted as an inpatient to Mission Hospital (abbreviated as “PMH” in the FAC) on 6/8/25. (FAC, ¶ 46.) Plaintiffs allege Defendants violated the Elder Abuse Act and/or ratified violations of the Act by “failing to provide JESSICA with adequate supervision and assistance to keep her safe and protect her from the negligent failure to appropriately treat, manage, care, and diagnose JESSICA before, during, and after her cardiac arrest in the Emergency Department at PMH MV on June 8, 2025, failing to provide her with adequate custodial care, and failing to protect her from health and safety hazards while she was in their care and custody, in violation of Welfare and Institutions Code § 15610.57.” (¶¶ 48-54.) Defendant contends this is a “straightforward wrongful death action” based on medical negligence, which should be subject to damage cap restrictions on medical malpractice claims under California’s Medical Injury Compensation Reform Act (“MICRA”). Defendant argues Plaintiffs are attempting to “inflate the value of this case” by seeking enhanced damages under the Elder Abuse Act when “the allegations in the FAC amount to medical negligence, if that...” (Motion, p. 1.) Specifically, Defendant contends Plaintiffs fail to allege specific facts showing “reckless, oppressive, fraudulent, or malicious conduct” involving physical abuse or neglect of decedent. (Motion, p. 3.) Defendant further argues this cause of action is “mutually exclusive and/or duplicative” of Plaintiff’s first claim for wrongful death. (Motion, p. 6.) Rather, Defendant asserts that at most, Plaintiff has alleged professional negligence falling within MICRA, not the heightened standard of abuse or neglect under the Elder Abuse Act. In opposition, Plaintiffs argue they adequately allege facts demonstrating dependent abuse or neglect, including the following
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		allegations in paragraphs 27-29 related to withdrawal of life support: “27. As result of negligent care before during and after her cardiac arrest in the Emergency Dept., JESSICA suffered severe permanent hypoxic-ischemic brain injury. 28. Despite multiple ECG’s performed after her cardiac arrest which showed cardiac ischemia, none of the physicians who treated JESSICA, including Dr. Quershi and Dr. Shakeel, diagnosed her cardiac ischemia or requested a cardiology consult. 29. Instead on June 23, 2025, at the specific request of Dr. Qureshi, the Ethics Committee recommended activation of the non-beneficial treatment policy and placed a DNR (Do Not Resuscitate) on JESSICA’s chart which was objected to by WILBERT, who was given 5 days to agree to turning off life support or JESSICA’s treating doctors would disconnect the ventilator over the objection of WILBERT.” Plaintiffs argue, “These are not allegations of mere negligent medical judgment. Plaintiffs expressly allege Defendant initiated a process to terminate life-sustaining treatment and withdraw ventilatory support from a dependent adult despite family objection. The threatened removal of life support constitutes withholding of necessary medical care and custodial protection sufficient to plead neglect under Welfare and Institutions Code section 15610.57.” (Opp. at p. 7:1-5.) In reply, Mission Hospital asserts that these allegations involve “professional decision-making” under the hospital’s standard ethics process implemented to avoid providing medically ineffective treatment, not “statutory neglect or abuse.” (Reply, pp. 2-3.) Moreover, there was no actual disconnection of life support or abandonment of care.
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	Defendant cites <i>Alexander v. Scripps Memorial Hospital La Jolla</i> (2018) 23 Cal.App.5th 206, 223, in which the appellate court upheld the sustained demurrer where the hospital allegedly delayed transfer to another hospital by approximately one day and “administered drugs to Elizabeth to hasten her death and withheld nutrition, hydration, and pain medication.” (Opp. at p. 3.) There, the appellate court held, “although Plaintiffs alleged Defendants failed to facilitate Elizabeth's transfer to another facility and withheld pain medication, nutrition, and fluids, the third amended complaint is replete with references to the extensive medical care Elizabeth received during her four-day hospitalization.” (<i>Ibid.</i>) At this stage, Defendant has not shown that <i>Alexander</i> is on point based on Plaintiff’s allegations that Defendant wrongfully attempted to withdraw life support. While Defendant may assert there were appropriate bases to make such a decision, the Court cannot consider such evidence at this stage. “[C]ourts may not turn a demurrer into a contested evidentiary matter by determining what the proper interpretation of the evidence is. A court errs when its decision reflects such a consideration of the evidence.” (<i>Panterra GP, Inc. v. Superior Court of Kern County</i> (2022) 74 Cal.App.5th 697, 711 [cleaned up].) “A plaintiff is permitted to plead alternative inconsistent theories.” (<i>Dubin v. Robert Newhall Chesebrough Trust</i> (2002) 96 Cal.App.4th 465, 477.) “Pleading alternative theories of recovery “does not run afoul of truthful pleading.” (<i>Panterra GP, Inc. v. Superior Court of Kern County</i> (2022) 74 Cal.App.5th 697, 711, fn. 10.) Here, liberally construing the complaint and drawing all reasonable inferences in Plaintiffs’ favor as the Court is required to do at this stage, Plaintiff has adequately alleged facts which could show that Mission Hospital engaged in reckless or conscious dependent abuse or neglect under Plaintiffs’ second cause of action. (See <i>Liapes v. Facebook, Inc., supra</i>, 95 Cal.App.5th at 919.) This includes Plaintiffs’ allegation that Mission Hospital attempted to wrongfully withdraw life support from decedent over the objections of
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		decedent's husband, when in fact decedent was able to survive for several months thereafter. At this stage, Plaintiffs are permitted to plead alternative theories of liability against moving Defendant. Tentative Ruling: Demurrer to First Amended Complaint ("FAC") by Defendant Mission Hospital Regional Medical Center dba Providence Mission Hospital Mission Viejo ("Mission Hospital") is OVERRULED.
8	Rivas vs. Hurtado 2021-01225380	Motion to Be Relieved as Counsel of Record Mr. Madoni has established good cause for counsel to withdraw. He asserts a conflict of interest has arisen that cannot be waived. Moving attorney has shown that he has complied with all the requirements of the California Rules of Court, Rule 3.1362 and filed all the required forms. Tentative Ruling: The motion of attorney Stephen A. Madoni to withdraw as attorney of record for Defendant/Cross-Complainant Susana Hurtado is GRANTED. (Code Civ. Proc. § 284, CRC 3.1362.) Attorney will be relieved as counsel of record for client effective upon filing of a proof of service of the signed order on client. Moving attorney is to give notice.
9	Sozer vs. Yoshiharu Global Co. 2024-01427440	Motion to Be Relieved as Counsel of Record Off Calendar